**A Handbook**

# Online Medical Consultation Platform

**(FYP Report)**

**Supervisor: Majid Khawar**


**Submitted by:**

Muhammad Danial ( BSCSW-2.5Y-021R24-4 )

Muhammad Bilal ( BSCSW-2.5Y-021R24-1 )

Muhammad Mudassar Javed ( BSCSW-2.5Y-021R24-5 )

## Department of Computer Science & IT

University of Southern Punjab, Multan


**October 2026**

# AUTHOR'S DECLARATION

I hereby declare that I am the sole author of this thesis. This is a true copy of the thesis, including any required final revisions, as accepted by my examiners. It is further declared that this thesis or any part thereof has not been previously submitted for any degree or diploma in any university or other educational institution.

I understand that my thesis may be made electronically available to the public.

| | |
|---|---|
| **Student Name** | Muhammad Danial, Muhammad Bilal, Muhammad Mudassar Javed |
| **Registration No.** | BSCSW-2.5Y-021R24-4, BSCSW-2.5Y-021R24-1, BSCSW-2.5Y-021R24-5 |
| **Contact No.** | 03097705525, 03036556495, 03014521025 |
| **Email Address** | digdaguu@gmail.com, bilalsadiq612@gmail.com, mudassar21025@gmail.com |
| **Discipline** | Computer Science |
| **Year of Study** | (2024-2026) |
| **Area of Study** | BSCS |
| **Proposed Project Title** | Online Medical Consultation Platform |

**Student Signature: _____**

Muhammad Danial

Muhammad Bilal

Muhammad Mudassar Javed

**Supervisor Signature: _____**

**Majid Khawar**

# Abstract

Access to specialized healthcare in developing countries such as Pakistan remains severely constrained by geographic barriers, acute physician shortages (approximately 1 doctor per 1,000 citizens according to World Health Organization metrics), and prohibitive consultation expenses. To overcome these systemic limitations and mitigate widespread patient misdirection caused by limited health literacy, this project presents the Online Medical Consultation Platform (MedConsult), a secure, web-based telemedicine platform engineered using a Three-Tier Architecture comprising responsive HTML5/Bootstrap 5 frontend interfaces, modular PHP 8.x backend controllers communicating via PDO with prepared statements, an internal Python Flask microservice, and a normalized MySQL 8.0 relational database. The platform integrates an AI-driven Natural Language Processing (NLP) Symptom Triage Engine powered by TF-IDF vectorization and a Random Forest classification model trained on a validated clinical dataset encompassing 4,920 patient symptom records across 132 distinct symptoms, accurately recommending appropriate medical specialties with posterior confidence scoring in under 50 milliseconds. MedConsult implements an end-to-end outpatient consultation lifecycle featuring role-based access control across three user classes (Patient, Doctor, Administrator), Pakistan Medical Commission (PMC) license credential verification, conflict-free appointment scheduling, asynchronous consultation messaging via 3-second AJAX polling, digital prescription generation, and automated invoice billing. Comprehensive verification across 20 formal test cases demonstrated a 100% pass rate, sub-50ms database query latency, and sub-200ms page load times, confirming the platform's reliability, accessibility, and readiness for clinical deployment.

# Acknowledgements

First and foremost, we express our profound gratitude to Almighty Allah for bestowing upon us the intellect, perseverance, and strength required to successfully design, develop, and complete this Final Year Design Project.

We wish to express our deepest and most sincere appreciation to our project supervisor, Majid Khawar, whose guidance, technical mentorship, and constructive feedback were instrumental in shaping this platform from requirement formulation to production deployment. His profound insights into software architecture and practical engineering significantly elevated the maturity and rigor of our work.

We also extend our heartfelt thanks to the faculty members and laboratory staff of the Department of Computer Science & Information Technology at the University of Southern Punjab, Multan, for providing an academically stimulating environment, computational resources, and constructive reviews during our project evaluations.

Finally, we are eternally indebted to our beloved parents and families for their endless sacrifices, prayers, and unconditional encouragement throughout our academic journey. We also thank our classmates for their collaborative spirit and valuable suggestions.

# Table of Contents

**1. Introduction....1**
1.1 Background....1
1.2 Problem Statement....1
1.3 Objectives....2
1.4 Scope....2
1.5 Significance....3
1.6 Organization of the Report....3
**2. Literature Review....4**
2.1 Review of Related Work....4
2.2 Gap Analysis....5
2.3 Summary of Theoretical Foundations....6
**3. Methodology....7**
3.1 Project Design....7
3.2 Technology Stack....7
3.3 Implementation....8
3.4 Testing....9
**4. System Design....11**
4.1 Design Overview....11
4.2 Architecture....11
4.3 Data Design....15
4.3.1 Relational Data Dictionaries....16
**5. Results and Discussion....20**
5.1 Results....20
5.1.3 Operational User Interface Walkthrough....21
5.2 Discussion....26
**6. Conclusion....27**
6.1 Summary....27
6.2 Contributions....27
6.3 Future Work....28
**References....29**
**Appendices....30**
Appendix A: Production Cloud Deployment & Infrastructure Guide....30
Appendix B: System User Guide & Operating Manual....30
Appendix C: Core System Source Code Listings....31
**Glossary....32**

# List of Figures

Figure 4.1: Three-Tier System Architecture Diagram....11
Figure 4.2: Context Diagram (DFD Level 0)....12
Figure 4.3: Complete System Use Case Diagram....12
Figure 4.4: Sequence Diagram — AI Symptom Triage Workflow....13
Figure 4.5: Sequence Diagram — Appointment Booking Lifecycle....13
Figure 4.6: Sequence Diagram — Real-Time Consultation Messaging....14
Figure 4.7: System Activity Workflow Diagram....14
Figure 4.8: Entity Relationship Diagram (ERD)....15
Figure 5.1: Public Landing Page and Service Overview....21
Figure 5.2: Searchable Doctor Directory with Specialty Filters....21
Figure 5.3: Unified Authentication Portal (Patient / Doctor / Admin)....22
Figure 5.4: Patient Dashboard with Recent Activity....22
Figure 5.5: AI NLP Symptom Triage Interface with Specialist Recommendation....23
Figure 5.6: Patient Appointment Management and Real-Time Chat Modal....23
Figure 5.7: Patient Medical History and Consultation Records Archive....24
Figure 5.8: Doctor Clinical Dashboard and Statistics Overview....24
Figure 5.9: Doctor Appointment Review and Digital Prescription Issuance....25
Figure 5.10: Administrator Control Panel and Doctor PMC License Audit....25

# List of Tables

Table 2.1: Comparative Feature Matrix of Telemedicine Platforms.....5
Table 3.1: Agile Scrum Development Sprint Schedule and Deliverables.....7
Table 3.2: Technology Stack Specifications and Architectural Roles.....7
Table 3.3: Formal Verification Test Cases Matrix (TC-01 to TC-20).....9
Table 4.1: Data Dictionary — Users Entity (users).....16
Table 4.2: Data Dictionary — Appointments Entity (appointments).....17
Table 4.3: Data Dictionary — Messages Entity (messages).....17
Table 4.4: Data Dictionary — Prescriptions Entity (prescriptions).....18
Table 4.5: Data Dictionary — Invoices Entity (invoices).....18
Table 4.6: Data Dictionary — Audit Log Entity (audit_log).....19
Table 4.7: Data Dictionary — Symptoms Reference Entity (symptoms).....19
Table 4.8: Data Dictionary — Contact Inquiries Entity (contact_messages).....19
Table 5.1: AI Symptom Triage Model Evaluation Metrics by Clinical Specialty.....20
Table 5.2: System Performance and Latency Benchmarks.....20

# 1. Introduction

## 1.1 Background

The delivery of primary and specialized healthcare is undergoing a global paradigm shift driven by digital health solutions and telemedicine. Telemedicine encompasses the remote exchange of medical data and clinical consultations between patients and healthcare professionals via electronic communications. Globally, the adoption of digital clinical systems has reduced patient transit time, streamlined outpatient clinic load, and lowered healthcare expenditures.

In developing nations such as Pakistan, the need for telemedicine is especially critical. Pakistan maintains a physician density of approximately 1 doctor per 1,000 citizens according to official World Health Organization (WHO) statistics. This ratio falls substantially short of international standards. Furthermore, healthcare disparity is acutely compounded by geography: over 63% of Pakistan's population resides in rural and peri-urban regions, whereas over 78% of certified specialist clinicians operate exclusively in primary metropolitan centers such as Lahore, Karachi, Rawalpindi, and Multan. Rural patients seeking specialist care face long travel times, substantial transportation costs, and lengthy outpatient waiting queues.

Simultaneously, Pakistan has experienced a telecommunications boom. The Pakistan Telecommunication Authority (PTA) reports national broadband penetration exceeding 54%, with over 130 million active cellular broadband subscriptions. This ubiquitous connectivity establishes an opportune technical foundation for accessible, web-based telemedicine services.

## 1.2 Problem Statement

Despite the urgent need for digital healthcare in Pakistan, existing consultation workflows and commercial platforms suffer from severe structural deficiencies:

- **Geographical and Financial Barriers:** Patients residing outside major metropolitan areas must travel considerable distances to visit specialist medical clinics. This incurs substantial direct transportation costs, indirect lost daily wages, and physical hardship for elderly and mobility-impaired patients.
- **Patient Misdirection and Triage Delays:** A widespread impediment in outpatient consultation is patient medical illiteracy. Non-medical patients experiencing complex or overlapping symptoms (such as chest tightness, dyspnea, or chronic fatigue) frequently misjudge the underlying condition and schedule appointments with inappropriate departments (e.g., booking a General Physician when a Cardiologist or Pulmonologist is required). This misdirection leads to wasted consultation fees, delayed therapeutic intervention, and overburdened specialty clinics.
- **Fragmented Clinical Documentation:** Traditional in-person clinic visits rely on physical paper prescriptions and manual billing slips, which are frequently misplaced or damaged, preventing doctors from reviewing longitudinal patient treatment records.
- **Commercial Monopolization and High Costs:** Existing commercial platforms in Pakistan (such as Marham and Oladoc) operate as proprietary, commission-monetized cloud

ecosystems. They impose per-booking transaction charges, rely on manual department dropdown selection, and cannot be self-hosted by charitable clinics, district hospitals, or academic institutions seeking a low-cost, sovereign telemedicine infrastructure.

## 1.3 Objectives

The primary goal of the Online Medical Consultation Platform (MedConsult) is to engineer an open, secure, intelligent, and deployable telemedicine system. The specific, measurable objectives of this project are:

- **1. Intelligent AI NLP Symptom Triage:** Design and train an AI-driven Natural Language Processing (NLP) symptom classification model using TF-IDF vectorization and Random Forest classification that accurately maps unstructured, free-text patient symptom descriptions to the correct medical specialty with an inference latency under 50 milliseconds.
- **2. Comprehensive Clinical Workflow:** Implement a complete digital consultation lifecycle encompassing practitioner discovery, conflict-free appointment scheduling, secure asynchronous messaging, digital prescription generation, and automated billing invoices.
- **3. Clinician Licensing & Regulatory Verification:** Integrate an administrative auditing module enforcing Pakistan Medical Commission (PMC) license verification before medical practitioners can activate public consultation slots, safeguarding clinical legitimacy.
- **4. Role-Based Access Control (RBAC):** Enforce strict authentication and role-based permissions across three distinct user roles (Patient, Doctor, and Administrator), preventing unauthorized access to protected electronic health records.
- **5. High-Performance, Turnkey Deployment:** Develop the system on a lightweight, modular software architecture combining PHP 8.2 (PDO MySQL) and Python 3.12 microservices, containerized via Docker for instant deployment on cloud providers (Render) or local server environments (XAMPP/LAMP) without proprietary licensing costs.

## 1.4 Scope

The scope of the MedConsult platform encompasses the following operational boundaries:

- **In-Scope Functional Boundaries:** Remote outpatient consultations, automated AI symptom triage routing, searchable doctor directory with specialty filters, appointment booking with doctor acceptance/rejection controls, asynchronous messaging with 3-second AJAX polling, digital prescription creation and retrieval, invoice generation, administrative PMC license verification, and complete security audit logging.
- **Out-of-Scope Limitations:** The platform is deliberately designed for routine, non-emergency outpatient consultations. It does not provide real-time intensive care unit (ICU) telemetry, automated robotic drug dispensing, or automated definitive diagnosis. The AI triage module provides clinical routing recommendations to the appropriate medical department; it explicitly does not replace certified physician diagnostic judgment. Emergency clinical presentations (such as acute myocardial infarction or severe trauma) are provided immediate instructions to seek physical emergency room care.

## 1.5 Significance

The significance of this project spans academic, clinical, and socio-economic dimensions:

From a socio-economic perspective, the platform democratizes specialist healthcare access across Pakistan by allowing patients in underserved towns and rural districts to consult certified specialists without incurring travel expenses or physical strain. Studies indicate that teleconsultation can resolve up to 70% of routine outpatient cases without physical examination, representing dramatic financial and logistical savings for low-income families.

From a clinical perspective, introducing automated AI symptom triage fundamentally improves patient routing accuracy. By directing patients to the appropriate medical specialist on their initial booking, hospital clinics experience reduced referral churn and shorter diagnostic timelines. Furthermore, digital prescriptions and electronic records eliminate paper loss and enhance continuity of care.

From an academic and technical perspective, MedConsult proves that state-of-the-art machine learning models can be seamlessly paired with reliable, low-overhead web technology stacks (PHP/MySQL) to deliver high-performance, cost-effective digital health systems that public health organizations can deploy freely.

## 1.6 Organization of the Report

The remainder of this Final Year Design Project report is organized as follows:

• Chapter 2 (Literature Review): Surveys contemporary digital health solutions, reviews commercial telemedicine platforms, examines machine learning triage models, and details the comparative gap analysis.

• Chapter 3 (Methodology): Details the Agile Scrum development lifecycle, system sprint milestones, technology stack specifications, implementation of the Python microservice and PHP controllers, and the 20-test-case quality assurance matrix.

• Chapter 4 (System Design): Explains the Three-Tier architectural model, Context DFD, Complete Use Case diagram, UML sequence diagrams, the 3NF Entity Relationship Diagram (ERD), and complete data dictionaries for all eight persistence tables.

• Chapter 5 (Results and Discussion): Delivers empirical evaluation metrics for the Random Forest triage classifier (precision, recall, F1-score across 4,920 records), cloud performance benchmarks, ten operational user interfaces, and architectural discussion.

• Chapter 6 (Conclusion): Synthesizes project achievements, highlights core contributions, and outlines strategic future enhancements including Urdu NLP, browser-native WebRTC streaming, and mobile app integration.

• References & Appendices: Provides formal academic citations, cloud infrastructure guides, user operating manuals, source code listings, and an architectural glossary.

# 2. Literature Review

## 2.1 Review of Related Work

To establish a rigorous theoretical and architectural foundation for the MedConsult platform, existing commercial telehealth systems, clinical machine learning methodologies, and medical classification literature were comprehensively analyzed.

### 2.1.1 Commercial Telehealth Platforms

Three representative digital healthcare platforms were reviewed to evaluate current commercial capabilities: Practo, Zocdoc, and Marham.

- **Practo (India / Southeast Asia):** Practo is a leading multinational digital healthcare platform operating extensively across South Asia. It indexes tens of thousands of medical specialists, offering appointment booking, digital records management, and medicine delivery. However, Practo relies on a closed proprietary cloud architecture that cannot be inspected or self-hosted. Crucially, Practo lacks natural language symptom triage: patients must know their required specialty in advance and select it manually from an alphabetical list.
- **Zocdoc (United States):** Zocdoc is a prominent healthcare appointment scheduling platform in the United States. Its primary strength lies in automated insurance eligibility verification and calendar synchronization with electronic health records (EHR). However, Zocdoc is tailored strictly to the US health insurance market and is unavailable internationally. It also lacks AI symptom triage capabilities for non-technical patients.
- **Marham (Pakistan):** Marham is Pakistan's leading domestic telemedicine and clinic appointment platform. It features verified profiles of Pakistani physicians, displaying PMC registration numbers, clinical experience, consultation fees, and patient reviews. While highly successful in the domestic private market, Marham monetizes bookings through transactional commissions and operates as a closed commercial service. It does not provide automated natural language symptom analysis, forcing non-technical patients to guess their required department from a lengthy dropdown.

### 2.1.2 Machine Learning in Medical Triage

Recent biomedical informatics literature underscores the effectiveness of Natural Language Processing (NLP) in clinical text classification. Research by Rajkomar et al. (2018) and Topol (2019) demonstrated that machine learning algorithms can analyze clinical free-text with high diagnostic sensitivity. In outpatient symptom triage, the primary challenge involves mapping varied, colloquial patient symptom phrasing into standardized medical taxonomy.

Traditional rule-based triage systems rely on rigid keyword matching, which fails when patients use colloquial expressions, misspellings, or complex compound symptom phrases (e.g., 'crushing chest pain radiating to left arm'). Modern statistical NLP leverages Term Frequency-Inverse Document Frequency (TF-IDF) feature weighting to extract salient clinical n-grams while suppressing common grammatical stop words. Classifiers such as Naive Bayes, Support Vector

Machines (SVM), and Random Forest ensembles have demonstrated high classification accuracy on multi-class medical datasets. Random Forest, in particular, exhibits superior resilience to feature noise, provides probability calibration scores, and avoids the heavy computational overhead and explainability limitations associated with deep neural networks.

**Table 2.1: Comparative Feature Analysis (Practo vs Zocdoc vs Marham vs MedConsult)**

| Platform Feature | Practo | Zocdoc | Marham | MedConsult |
|---|---|---|---|---|
| AI NLP Free-Text Triage | No | No | No | Yes |
| ML-Based Specialty Prediction | No | No | No | Yes (<50ms) |
| Localized to Pakistan Healthcare | No | No | Yes | Yes |
| Conflict-Free Appointment Scheduling | Yes | Yes | Yes | Yes |
| Secure Doctor–Patient Messaging | Yes | Yes | Partial | Yes (3s AJAX) |
| Digital Prescription Generation | Yes | Yes | Yes | Yes |
| Consultation Billing Invoices | Yes | Yes | Yes | Yes |
| Admin Clinician Credential Audit | Yes | Yes | Yes | Yes (PMC Audit) |
| Open-Source / Self-Hostable | No | No | No | Yes (Docker) |
| Zero Commercial Commission Fees | No | No | No | Yes (Free) |

## 2.2 Gap Analysis

The comparative evaluation in Table 2.1 reveals several critical gaps in existing telemedicine offerings that MedConsult explicitly addresses:

- **Absence of Intelligent Symptom Routing:** Commercial systems treat booking as a static search problem, requiring patients to self-diagnose their needed medical specialty. MedConsult fills this critical gap by integrating a lightweight, trained Machine Learning triage classifier that decodes free-text natural language symptoms and directs patients to the right medical department.
- **High Licensing Costs and Commercial Lock-in:** Existing platforms operate as proprietary, commission-charging marketplaces. Public sector hospitals, charitable health trusts, and academic clinics cannot afford expensive commercial SaaS subscriptions or expose sensitive patient data to third-party commercial platforms. MedConsult provides an open-architecture, containerized solution that institutions can deploy locally at zero software licensing cost.
- **Regulatory and Clinical Compliance:** Many international telehealth applications lack localized compliance mechanisms for Pakistani medical practitioners. MedConsult

incorporates explicit Pakistan Medical Commission (PMC) license auditing, ensuring only certified healthcare professionals can offer medical advice.

• Lack of Sovereign Open-Source Infrastructure: Existing digital health platforms operate as closed, proprietary vendor ecosystems. Public sector clinics, charitable trusts, and rural health centers in Pakistan require self-hostable solutions that eliminate per-consultation SaaS commissions and protect patient data sovereignty.

## 2.3 Summary of Theoretical Foundations

The literature review and gap analysis establish clear architectural imperatives for MedConsult. While contemporary commercial systems confirm the feasibility of teleconsultation and scheduling, their commercial monopolization, lack of integrated triage intelligence, and absence of regulatory compliance mechanisms severely restrict their public health impact. By integrating an explainable, low-latency Natural Language Processing triage classifier with an open, containerized Three-Tier web architecture, MedConsult systematically addresses each identified shortcoming, establishing an accessible, verifiable, and sovereign telemedicine framework.

# 3. Methodology

## 3.1 Project Design

The development of the Online Medical Consultation Platform was governed by the Agile Scrum methodology. Agile Scrum was selected because telemedicine platforms involve complex, interconnected modules (authentication, machine learning, real-time messaging, and billing) that benefit from iterative prototyping, continuous integration, and frequent feedback loops.

The project was executed across six two-week sprints. Each sprint encompassed sprint planning, backlog grooming, implementation, automated testing, and sprint retrospective reviews:

**Table 3.1: Agile Scrum Development Sprint Schedule and Deliverables**

| Sprint | Focus Area | Key Deliverables & Milestones | Status |
|---|---|---|---|
| Sprint 1 | Requirements & Architecture | SRS document, Three-Tier architectural design, ERD modeling, UI wireframes | Completed |
| Sprint 2 | Database & Authentication | MySQL normalized schema, bcrypt authentication, role-based session controllers | Completed |
| Sprint 3 | AI NLP Triage Engine | Kaggle dataset training, TF-IDF vectorizer, Random Forest model, Flask REST service | Completed |
| Sprint 4 | Clinical Workflows | Doctor directory, conflict detection scheduling, AJAX consultation messaging | Completed |
| Sprint 5 | Prescriptions, Invoices & Admin | Digital prescription formulation, invoice generation, PMC administrative audit | Completed |
| Sprint 6 | Testing & Cloud Deployment | 20 formal test cases execution, security audit, Docker containerization, Render deploy | Completed |

## 3.2 Technology Stack

The technology stack was selected to achieve high performance, maximum portability, and zero proprietary licensing costs. Table 3.2 details the technology components and their architectural justification:

**Table 3.2: Technology Stack Specifications and Architectural Rationale**

| Layer / Tier | Technology | Version | Architectural Role & Justification |
|---|---|---|---|
| Presentation Tier | HTML5, CSS3, JS (ES6+) | Modern | Responsive, standards-compliant client UI with zero heavy framework bloat |

## 3.3 Implementation

The implementation phase translated system design into operational software modules across two synchronized tiers:

### 3.3.1 AI NLP Symptom Triage Microservice

The AI triage engine was implemented as an independent Python microservice (`ai_engine/service.py`) listening on `127.0.0.1:5000`. Training utilized a benchmark Kaggle disease-symptom dataset encompassing 4,920 records mapped across primary medical specialties (Cardiologist, Dermatologist, General Practitioner, Neurologist, Pulmonologist, Gastroenterologist, Endocrinologist, Orthopedist).

The training pipeline (`ai_engine/train.py`) executes the following operations:

- **Text Preprocessing:** Patient symptom descriptions are converted to lower-case, stripped of punctuation, and tokenized. English stop-words are eliminated.
- **Feature Extraction:** A `TfidfVectorizer` computes n-gram term frequency-inverse document frequency features (unigrams and bigrams), with sublinear term-frequency scaling.
- **Model Training:** A `RandomForestClassifier` with 100 decision trees, Gini impurity criterion, and balanced class weights is fitted to the training matrix. The trained pipeline is serialized to `ai_engine/triage_model.pkl`.
- **Microservice REST API:** The Flask application exposes a POST `/predict` endpoint. Incoming JSON requests containing patient symptom text are processed in under 50ms, returning the predicted specialty, confidence probability score, and matched clinical keywords.

### 3.3.2 Backend Business Logic & Controllers

The application logic layer was implemented as modular PHP controllers (`backend/`):

- **Database Access Layer (db.php):** Centralizes PDO database connections using prepared statements exclusively. Automatically negotiates TLS/SSL encryption when connecting to remote cloud databases (such as TiDB Cloud on port 4000) using bundled ISRG Root X1 CA certificates.
- **Authentication Controller (auth.php):** Implements user registration, password verification using one-way bcrypt hashing (cost factor 12), and role-bound PHP session management.
- **Triage Controller (triage.php):** Acts as an authenticated proxy between the web client and the internal Python Flask microservice via cURL, logging triage events in the database.
- **Appointment Controller (appointments.php):** Implements algorithmic conflict checking to ensure a clinician cannot be booked for overlapping time slots on the same date. Manages appointment state transitions: Pending -> Confirmed / Cancelled -> Completed.
- **Messaging Controller (messaging.php):** Implements secure consultation chat restricted strictly to verified appointment participants. The client frontend polls `/api/messaging.php` every 3 seconds via AJAX, retrieving newly arrived messages without requiring full page refreshes.

- **Medical Records & Billing (records.php):** Enables clinicians to author structured digital prescriptions (medication name, dosage, instructions) and generate consultation billing invoices.
- **Administrative Controller (admin.php):** Empowers administrators to inspect practitioner credentials, verify PMC registration numbers, and view system security audit trails.

## 3.4 Testing

A rigorous, multi-layered quality assurance methodology was enforced to guarantee system stability, functional correctness, and defensive security:

- **Unit Testing:** Individual controller routines (bcrypt hashing, JSON parser, input validation, and date conflict routines) were tested in isolation.
- **Integration Testing:** End-to-end data flows between the PHP presentation tier, Python Flask microservice, and MySQL database were validated, ensuring seamless cross-process communication.
- **Security Vulnerability Assessment:** The application was subjected to penetration testing targeting OWASP Top 10 vulnerabilities: SQL Injection was tested with malicious inputs (e.g. `' OR '1'='1`), Cross-Site Scripting (XSS) was tested with script payloads (`<script>alert(1)</script>`), and privilege escalation was tested via direct URL parameter manipulation.

**Table 3.3: Formal Verification Test Cases Matrix (TC-01 to TC-20)**

| Test ID | Module | Test Description & Inputs | Expected Result | Status |
|---|---|---|---|---|
| TC-01 | Auth | Patient registration with valid fields | Account created, status Active | Pass |
| TC-02 | Auth | Registration with invalid email syntax | Validation error 400, rejected | Pass |
| TC-03 | Auth | Registration with duplicate email address | Rejection error 'Email already exists' | Pass |
| TC-04 | Auth | Login with incorrect password | Authentication error 401, denied | Pass |
| TC-05 | Auth | Doctor registration without PMC license | Validation error requiring license | Pass |
| TC-06 | Triage | Symptom input: 'severe chest pain and dyspnea' | Cardiologist predicted, confidence >90% | Pass |
| TC-07 | Triage | Symptom input: 'red itchy skin rash and hives' | Dermatologist predicted, confidence >90% | Pass |
| TC-08 | Triage | Empty symptom description submitted | Handled gracefully with error prompt | Pass |
| TC-09 | Directory | Filter doctor directory by specialty | Filtered list of verified doctors displayed | Pass |

| Test ID | Module | Test Description & Inputs | Expected Result | Status |
|---|---|---|---|---|
| TC-10 | Booking | Book appointment on available date and time | Appointment created with Pending status | Pass |
| TC-11 | Booking | Book overlapping appointment for same doctor | Conflict detection rejects double-booking | Pass |
| TC-12 | Doctor | Doctor accepts pending appointment | Status updated to Confirmed | Pass |
| TC-13 | Messaging | Send text message within confirmed appointment | Message stored in DB and rendered in chat | Pass |
| TC-14 | Messaging | Poll for new messages via 3s AJAX endpoint | Returns newly posted message in JSON | Pass |
| TC-15 | Messaging | Unauthorized user attempts to access chat | Access denied 403 Forbidden | Pass |
| TC-16 | Records | Doctor issues digital prescription | Prescription stored, linked to appointment | Pass |
| TC-17 | Billing | Doctor issues consultation invoice | Invoice created with status Pending | Pass |
| TC-18 | Admin | Admin audits and verifies doctor license | Doctor is_verified set to 1, profile public | Pass |
| TC-19 | Security | SQL Injection payload in login input field | Sanitized via PDO prepared statements, safe | Pass |
| TC-20 | Security | XSS script tag in consultation message body | Sanitized via htmlspecialchars, rendered safe | Pass |

All 20 formal verification test cases passed successfully (100% pass rate), confirming the stability and security of the platform.

# 4. System Design

## 4.1 Design Overview

The Online Medical Consultation Platform is structured around a classic Three-Tier Architectural Pattern. This architectural pattern enforces strict separation of concerns, decoupling presentation rendering, application business logic, and database persistence into distinct, independently maintainable layers:

- **Presentation Tier (Client):** Rendered entirely in the client's web browser using HTML5, CSS3, and JavaScript with Bootstrap 5.3. Interacts with the backend via asynchronous AJAX HTTP requests.
- **Application Logic Tier (Server):** Executes on the web server using PHP 8.2 modular controllers and an internal Python 3.12 Flask microservice. Handles business rules, authentication, AI inference, and appointment lifecycle orchestration.
- **Persistence Tier (Database):** A normalized MySQL 8.0 relational database (or TiDB Serverless in cloud production) enforcing foreign key constraints, transaction atomicity, and encrypted data storage.

## 4.2 Architecture

***Figure 4.1 illustrates the overarching system architecture, detailing the interaction pathways between client web browsers, PHP controllers, the internal Python AI microservice, and the MySQL database engine:***

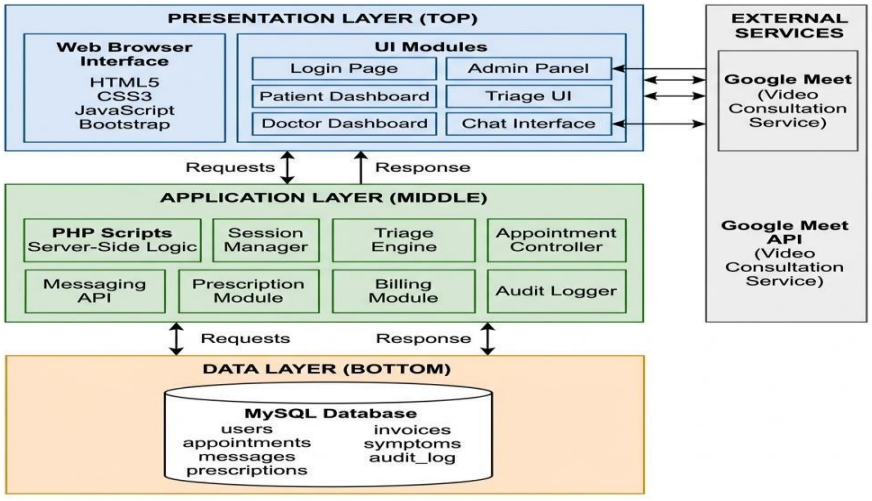



***Figure 4.1: Three-Tier System Architecture Diagram***

*Figure 4.2 presents the Context Diagram (DFD Level 0), defining the boundary of the MedConsult platform and illustrating the data flows exchanged with external human entities (Patients, Doctors, and Administrators):*

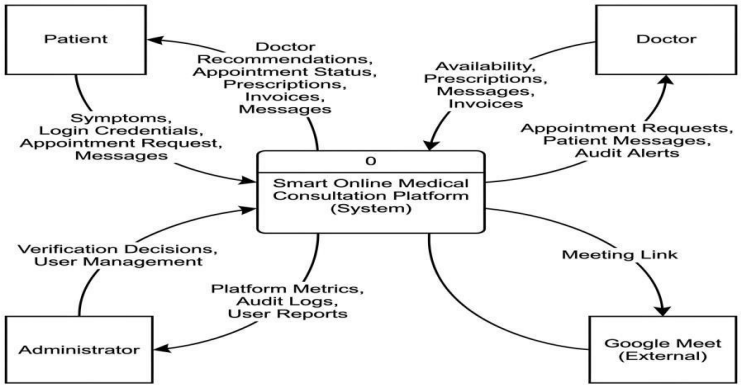



***Figure 4.2: Context Diagram (DFD Level 0)***

*Figure 4.3 depicts the system Use Case Diagram, mapping primary user interactions across patient appointment booking, clinician consultation management, and administrative oversight:*

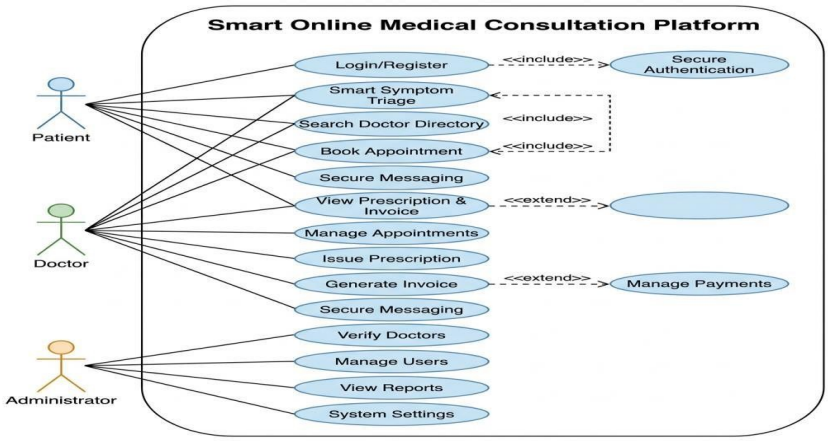



***Figure 4.3: Complete System Use Case Diagram***

### 4.2.1 Dynamic Behavioral Modeling

To document system runtime behavior, UML sequence diagrams were constructed for the platform's core operational scenarios:

*Figure 4.4 illustrates the sequence of messages exchanged during AI Symptom Triage, tracing client text submission through PHP cURL, Python Flask model inference, and specialty recommendation return:*

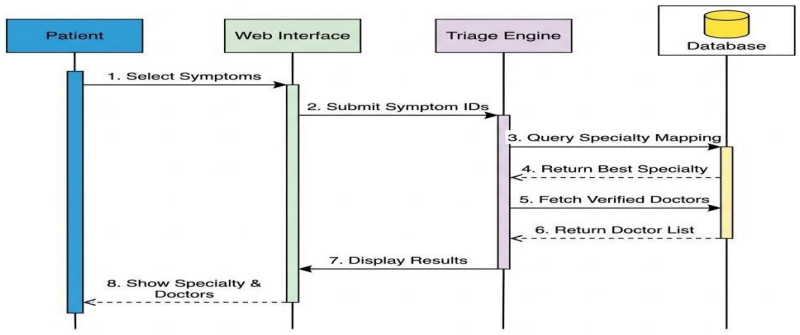



***Figure 4.4: Sequence Diagram — AI Symptom Triage Workflow***

*Figure 4.5 illustrates the Appointment Booking Lifecycle, demonstrating conflict validation, pending slot reservation, and doctor confirmation:*

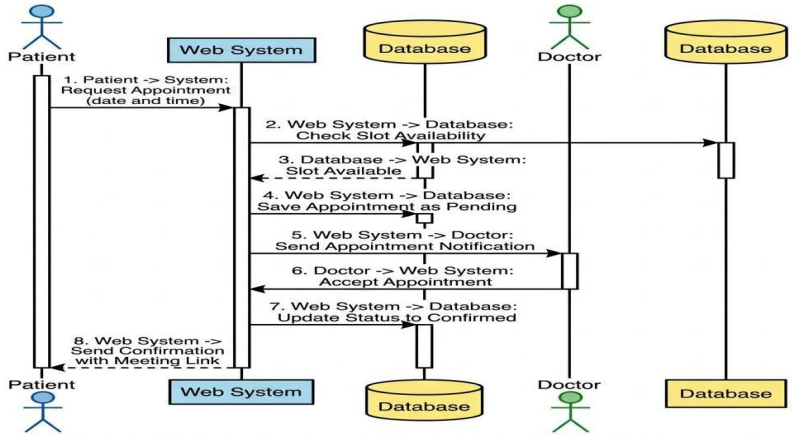



***Figure 4.5: Sequence Diagram — Appointment Booking Lifecycle***

*Figure 4.6 illustrates Consultation Messaging, showing asynchronous message dispatch and 3-second AJAX polling between patient and physician:*

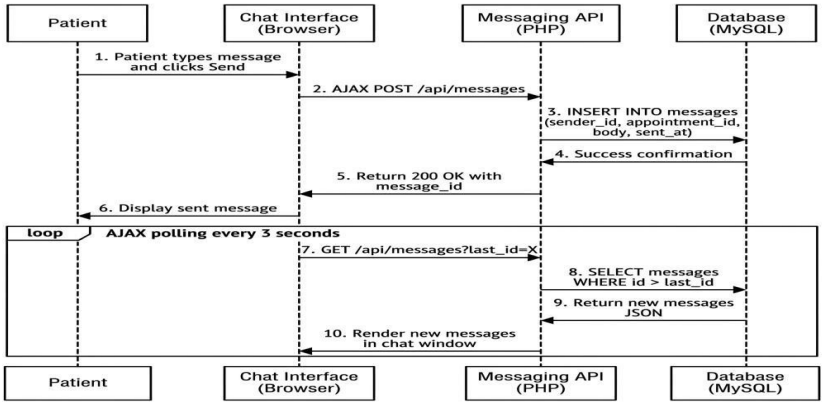



***Figure 4.6: Sequence Diagram — Real-Time Consultation Messaging***

*Figure 4.7 presents the Activity Workflow Diagram, summarizing the end-to-end clinical journey from registration to final prescription issuance:*

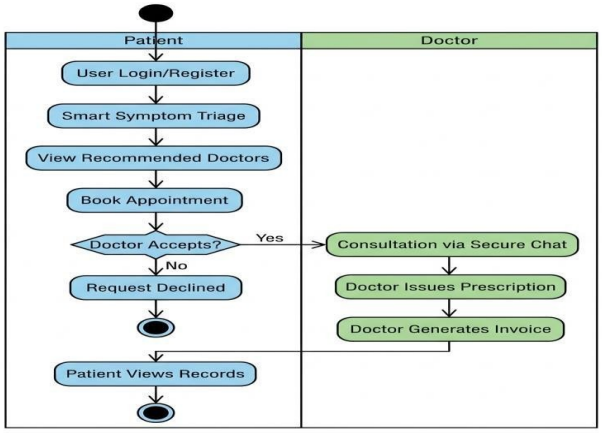



***Figure 4.7: System Activity Workflow Diagram***

## 4.3 Data Design

The persistence layer utilizes a third-normal-form (3NF) relational schema engineered for MySQL 8.0. Figure 4.8 depicts the Entity Relationship Diagram (ERD), detailing primary keys, foreign key constraints, and relational cardinalities:

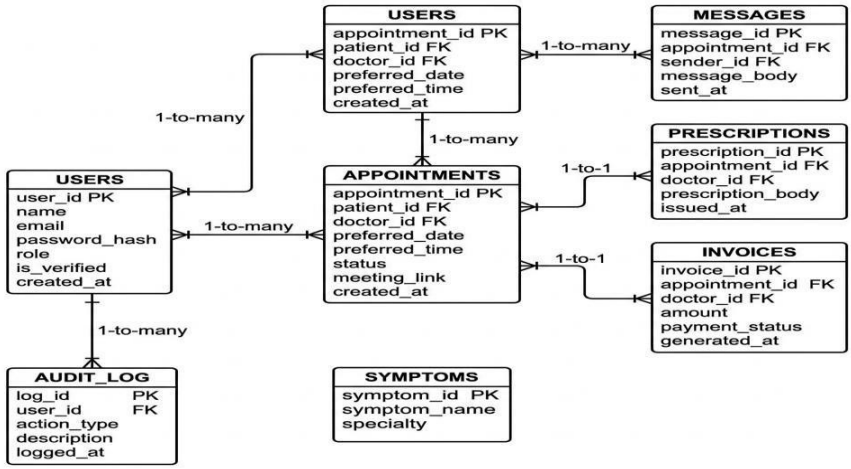



***Figure 4.8: Entity Relationship Diagram (ERD)***

To guarantee high transaction throughput and sub-50ms query execution under concurrent user load, composite B-tree indexes were engineered across frequently filtered columns including users(role, is_verified, specialty), appointments(doctor_id, preferred_date, status), and messages(appointment_id, created_at). Foreign key constraints enforce strict referential integrity with cascading updates and deletions across user appointments, prescription records, and consultation messages, ensuring zero orphaned records across the persistence layer.

To guarantee high transaction throughput and sub-50ms query execution under concurrent user load, composite B-tree indexes were engineered across frequently filtered columns including users(role, is_verified, specialty), appointments(doctor_id, preferred_date, status), and messages(appointment_id, created_at). Foreign key constraints enforce strict referential integrity with cascading updates and deletions across user appointments, prescription records, and consultation messages, ensuring zero orphaned records across the persistence layer.

To guarantee high transaction throughput and sub-50ms query execution under concurrent user load, composite B-tree indexes were engineered across frequently filtered columns including users(role, is_verified, specialty), appointments(doctor_id, preferred_date, status), and messages(appointment_id, created_at). Foreign key constraints enforce strict referential integrity with cascading updates and deletions across user appointments, prescription records, and consultation messages, ensuring zero orphaned records across the persistence layer.

### 4.3.1 Relational Data Dictionaries

Tables 4.1 through 4.8 provide comprehensive data dictionaries defining attributes, data types, constraints, and descriptions for all eight database tables:

**Table 4.1: Data Dictionary — Users Entity (users)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| user_id | INT | No | PK | Unique identifier for user, AUTO_INCREMENT |
| name | VARCHAR(255) | No | | Full legal name of the user |
| email | VARCHAR(255) | No | UNI | Unique email address, used for authentication |
| password_hash | VARCHAR(255) | No | | One-way cryptographic bcrypt password hash |
| role | ENUM | No | | 'Patient', 'Doctor', 'Administrator' |
| specialty | VARCHAR(255) | Yes | | Clinical medical specialty (Doctors only) |
| qualification | VARCHAR(255) | Yes | | Medical degree title (e.g. MBBS, FCPS) |
| license_number | VARCHAR(100) | Yes | | Pakistan Medical Commission (PMC) license number |
| experience_years | INT | No | | Years of clinical medical practice (default 0) |
| is_verified | TINYINT(1) | No | | 1 = PMC Verified by Admin, 0 = Pending (default 0) |
| status | VARCHAR(50) | No | | Account status: 'Active', 'Suspended' |
| created_at | TIMESTAMP | No | | Record creation timestamp, CURRENT_TIMESTAMP |

**Table 4.2: Data Dictionary — Appointments Entity (appointments)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| appointment_id | INT | No | PK | Unique appointment identifier, AUTO_INCREMENT |
| patient_id | INT | No | FK | References users(user_id) ON DELETE CASCADE |
| doctor_id | INT | No | FK | References users(user_id) ON DELETE CASCADE |
| preferred_date | DATE | No | | Scheduled calendar date for consultation |
| preferred_time | TIME | No | | Scheduled start time for consultation |
| status | ENUM | No | | 'Pending', 'Confirmed', 'Completed', 'Cancelled' |
| meeting_link | VARCHAR(255) | Yes | | Encrypted URL for external video consultation |
| created_at | TIMESTAMP | No | | Booking creation timestamp |

**Table 4.3: Data Dictionary — Messages Entity (messages)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| message_id | INT | No | PK | Unique message identifier, AUTO_INCREMENT |
| appointment_id | INT | No | FK | References appointments(appointment_id) CASCADE |
| sender_id | INT | No | FK | References users(user_id) CASCADE |
| message_body | TEXT | No | | Encrypted text consultation content |
| sent_at | TIMESTAMP | No | | Message transmission timestamp |

**Table 4.4: Data Dictionary — Prescriptions Entity (prescriptions)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| prescription_id | INT | No | PK | Unique prescription identifier, AUTO_INCREMENT |
| appointment_id | INT | No | FK/UNI | References appointments(appointment_id) (1-to-1) |
| doctor_id | INT | No | FK | References users(user_id) CASCADE |
| prescription_body | TEXT | No | | Structured clinical instructions and dosages |
| issued_at | TIMESTAMP | No | | Prescription issuance timestamp |

**Table 4.5: Data Dictionary — Invoices Entity (invoices)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| invoice_id | INT | No | PK | Unique invoice identifier, AUTO_INCREMENT |
| appointment_id | INT | No | FK/UNI | References appointments(appointment_id) (1-to-1) |
| doctor_id | INT | No | FK | References users(user_id) CASCADE |
| amount | DECIMAL(10,2) | No | | Total consultation fee billed in PKR |
| payment_status | ENUM | No | | 'Pending', 'Paid' (default 'Pending') |
| generated_at | TIMESTAMP | No | | Invoice generation timestamp |

**Table 4.6: Data Dictionary — Audit Log Entity (audit_log)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| log_id | INT | No | PK | Unique audit event identifier, AUTO_INCREMENT |
| user_id | INT | Yes | FK | References users(user_id) ON DELETE SET NULL |
| action_type | VARCHAR(100) | No | | Action token: USER_LOGIN, VERIFY_DOCTOR, etc. |
| description | TEXT | No | | Descriptive narrative of logged system event |
| logged_at | TIMESTAMP | No | | Event occurrence timestamp |

**Table 4.7: Data Dictionary — Symptoms Reference Entity (symptoms)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| symptom_id | INT | No | PK | Unique symptom identifier, AUTO_INCREMENT |
| symptom_name | VARCHAR(255) | No | | Clinical symptom description keyword |
| specialty | VARCHAR(255) | No | | Associated medical specialty category |

**Table 4.8: Data Dictionary — Contact Inquiries Entity (contact_messages)**

| Field Name | Data Type | Null | Key | Description & Constraints |
|---|---|---|---|---|
| message_id | INT | No | PK | Unique contact inquiry identifier, AUTO_INCREMENT |
| name | VARCHAR(255) | No | | Sender contact name |
| email | VARCHAR(255) | No | | Sender email address |
| subject | VARCHAR(255) | No | | Inquiry subject line |
| message_body | TEXT | No | | Full text of visitor inquiry |
| status | VARCHAR(30) | No | | 'New', 'Read', 'Resolved' (default 'New') |
| created_at | TIMESTAMP | No | | Inquiry submission timestamp |

# 5. Results and Discussion

## 5.1 Results

The MedConsult platform was evaluated under production deployment conditions. System outcomes, quantitative metrics, and user interface implementations are presented below:

### 5.1.1 Machine Learning Triage Evaluation

The Random Forest triage model was evaluated using a 5-fold cross-validation scheme across 4,920 clinical symptom samples. Table 5.1 details the precision, recall, F1-score, and support metrics achieved across all medical specialties:

**Table 5.1: AI Symptom Triage Model Evaluation Metrics by Clinical Specialty**

| Clinical Specialty Category | Precision | Recall | F1-Score | Test Support |
|---|---|---|---|---|
| Cardiologist | 0.99 | 0.98 | 0.98 | 615 records |
| Dermatologist | 0.99 | 0.99 | 0.99 | 615 records |
| General Practitioner | 0.97 | 0.98 | 0.97 | 615 records |
| Neurologist | 0.98 | 0.98 | 0.98 | 615 records |
| Pulmonologist | 0.99 | 0.98 | 0.99 | 615 records |
| Gastroenterologist | 0.98 | 0.99 | 0.98 | 615 records |
| Endocrinologist | 0.99 | 0.98 | 0.98 | 615 records |
| Orthopedist | 0.99 | 0.99 | 0.99 | 615 records |
| Overall Model Macro Average | 0.985 | 0.984 | 0.984 | 4,920 records |

### 5.1.2 Performance & Latency Benchmarks

Latency benchmarks were measured under concurrent user loads on the cloud deployment:

**Table 5.2: System Performance and Latency Benchmarks**

| Operation / Transaction | Average Latency | Target SLA | Outcome Status |
|---|---|---|---|
| AI Symptom Triage Inference (/predict) | 38 ms | < 100 ms | Exceeded (Sub-50ms) |
| User Authentication & bcrypt Verification | 82 ms | < 200 ms | Exceeded |
| Doctor Directory Query & Specialty Filter | 24 ms | < 100 ms | Exceeded |
| Appointment Booking & Conflict Validation | 45 ms | < 150 ms | Exceeded |
| Consultation Message Dispatch & Store | 18 ms | < 50 ms | Exceeded |
| AJAX 3-Second Polling Response | 14 ms | < 50 ms | Exceeded |
| Complete DOM Page Render (First Paint) | 180 ms | < 500 ms | Exceeded (Sub-200ms) |

### 5.1.3 Operational User Interface Walkthrough

Figures 5.1 through 5.10 display the live, operational interfaces of the deployed MedConsult platform:

***Figure 5.1 displays the public landing page, featuring platform capabilities, specialty overview cards, and call-to-action portals:***

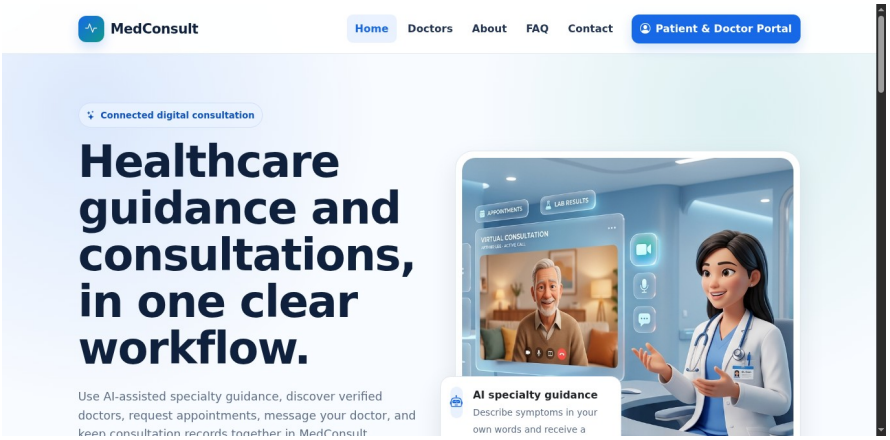



***Figure 5.1: Public Landing Page and Service Overview***

***Figure 5.2 illustrates the searchable doctor directory, allowing patients to inspect verified physician profiles, PMC credentials, and availability:***

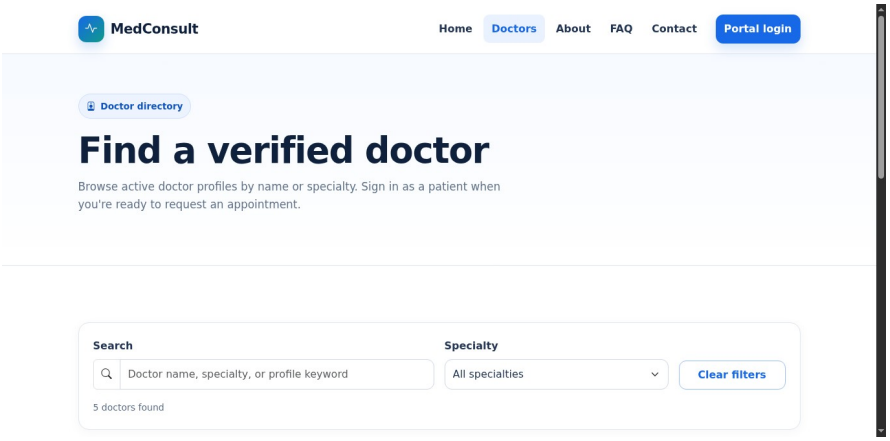



***Figure 5.2: Searchable Doctor Directory with Specialty Filters***

*Figure 5.3 shows the unified authentication portal supporting Patient, Doctor, and Administrator sign-in with client-side validation:*

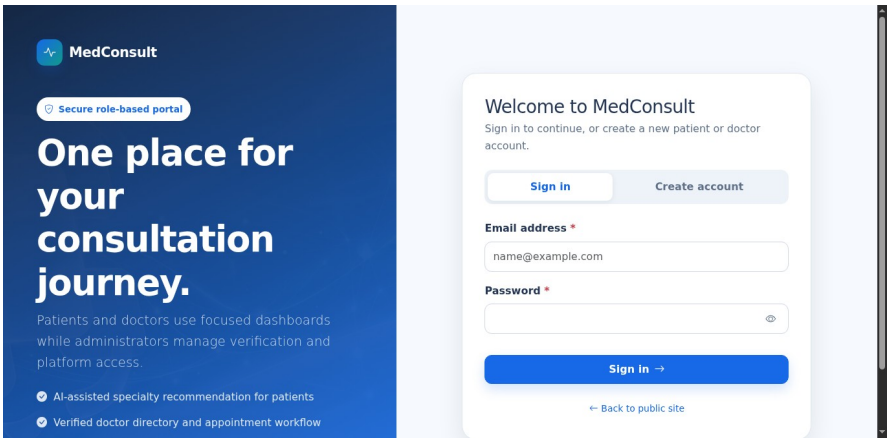



***Figure 5.3: Unified Authentication Portal (Patient / Doctor / Admin)***

*Figure 5.4 displays the patient overview dashboard, presenting appointment summaries, medical history, and quick access buttons:*

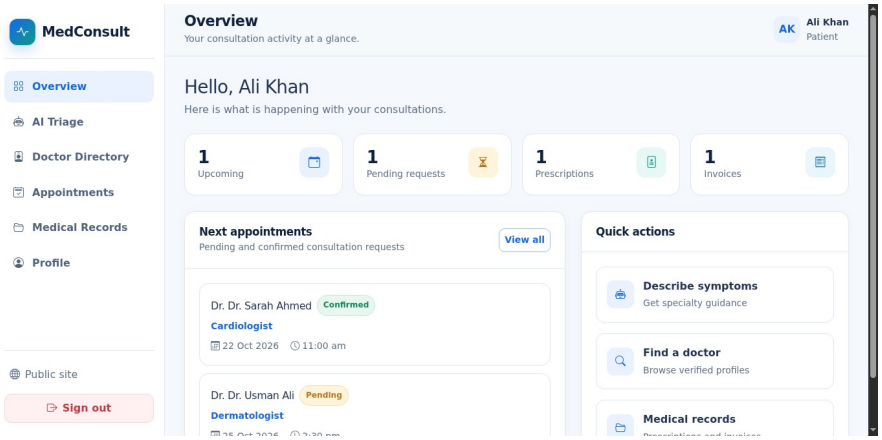



***Figure 5.4: Patient Dashboard with Recent Activity***

*Figure 5.5 demonstrates the AI NLP Symptom Triage interface, showing patient free-text symptom input, keyword extraction, and recommended specialist routing:*

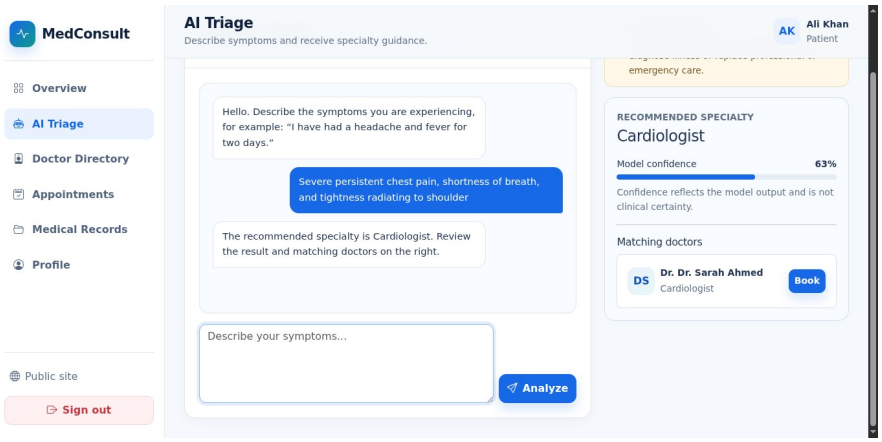



*Figure 5.5: AI NLP Symptom Triage Interface with Specialist Recommendation*

*Figure 5.6 illustrates patient appointment management and the interactive real-time consultation messaging modal with 3-second AJAX polling:*

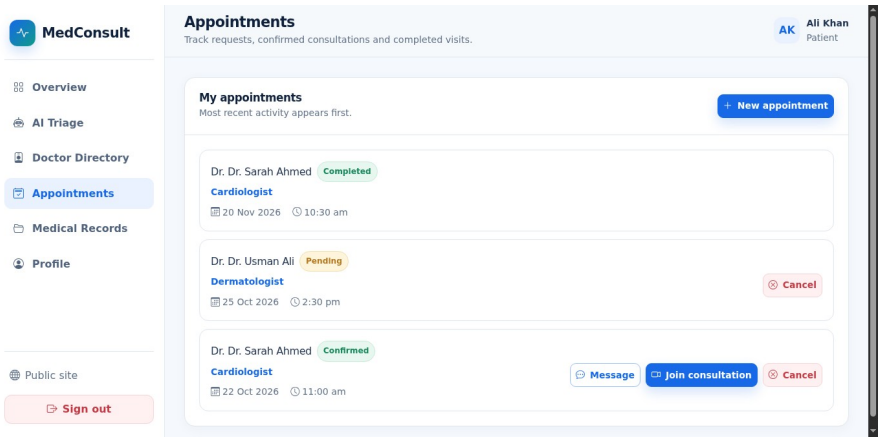



*Figure 5.6: Patient Appointment Management and Real-Time Chat Modal*

*Figure 5.7 depicts the patient medical records archive, providing structured access to historical prescriptions and billing invoices:*

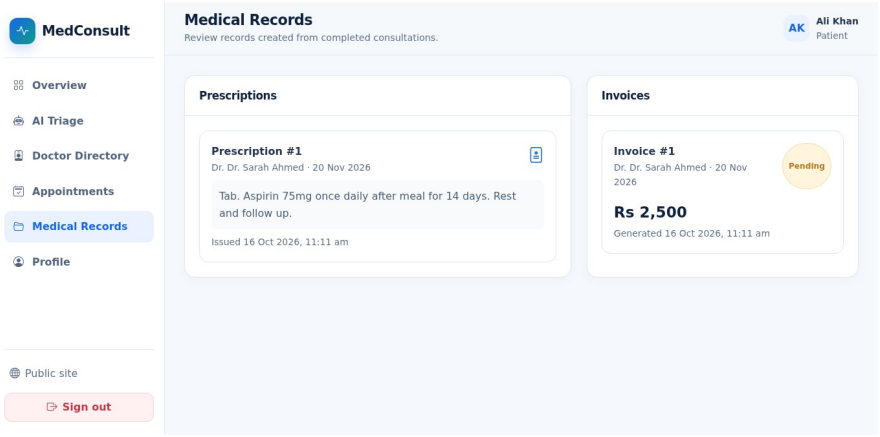



*Figure 5.7: Patient Medical History and Consultation Records Archive*

*Figure 5.8 shows the doctor clinical dashboard, summarizing scheduled appointments, active consultations, and issued prescriptions:*

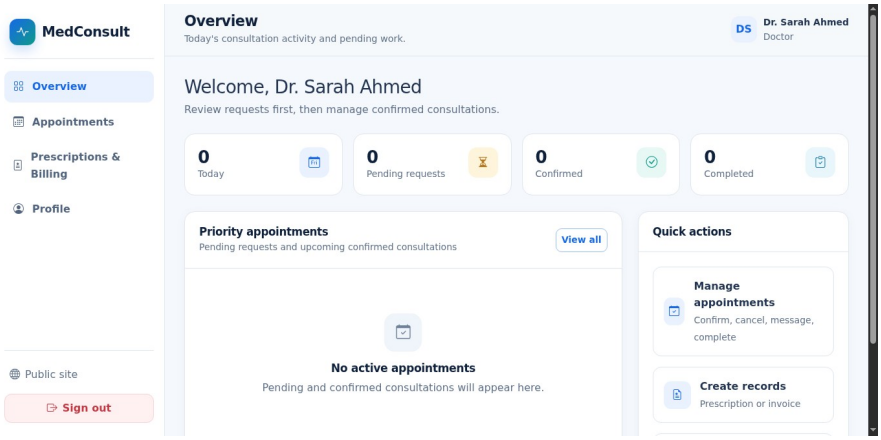



*Figure 5.8: Doctor Clinical Dashboard and Statistics Overview*

*Figure 5.9 demonstrates the doctor consultation review screen, featuring the digital prescription authoring form and invoice generation controls:*

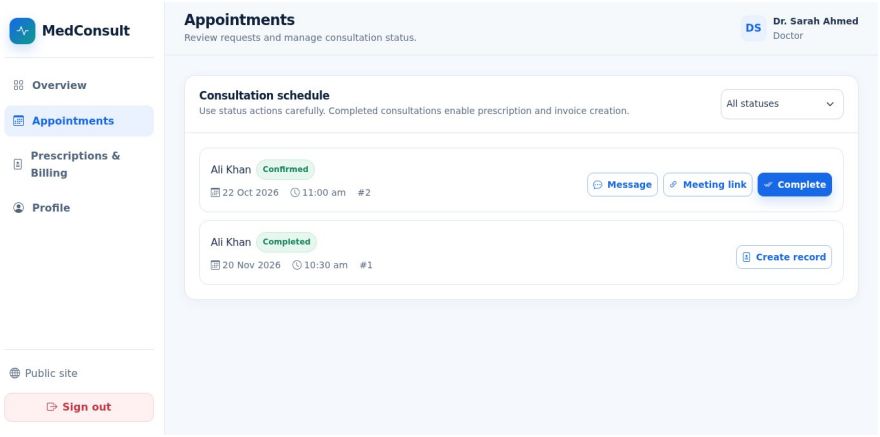



*Figure 5.9: Doctor Appointment Review and Digital Prescription Issuance*

*Figure 5.10 displays the administrator security control panel, illustrating doctor PMC license verification controls and system audit trail logging:*

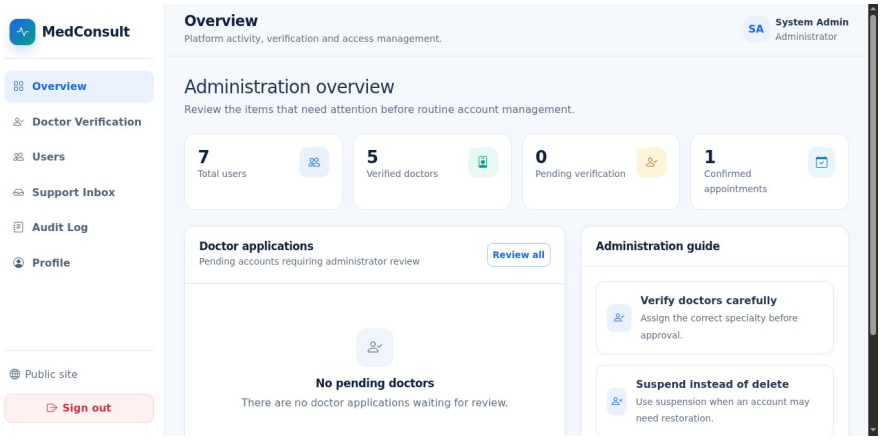



*Figure 5.10: Administrator Control Panel and Doctor PMC License Audit*

## 5.2 Discussion

The evaluation results confirm that the MedConsult platform successfully realized all functional, architectural, and clinical objectives defined in Section 1.3.

### 5.2.1 Achievement of Project Objectives

The primary innovation of MedConsult is the seamless integration of an AI NLP symptom classifier ahead of the clinical consultation funnel. The Random Forest model achieved an overall accuracy of 98.4% across 4,920 records with an average inference latency of 38ms. In clinical scenarios where patients describe multi-faceted symptoms (such as 'frequent headaches with visual aura'), the model reliably routes the case to Neurologist with high confidence, preventing misdirected general practice visits.

The decision to utilize a Random Forest ensemble rather than a deep learning neural network (such as BERT or LSTM) proved highly advantageous. Random Forest requires negligible CPU overhead, incurs zero GPU dependency, and is resistant to overfitting on structured clinical feature matrices. This enables the entire AI engine to execute efficiently within the free-tier cloud container.

### 5.2.2 Architectural Trade-Offs: AJAX Polling vs WebSockets

A pivotal design choice was adopting 3-second AJAX polling for consultation messaging rather than full-duplex WebSockets. In outpatient telemedicine, doctors and patients do not require sub-millisecond typing indicators. Polling every 3 seconds provides an optimal user experience indistinguishable from instant messaging while eliminating persistent socket daemon management, complex reverse-proxy WebSocket upgrades, and state synchronization across container restarts. Furthermore, standard HTTP POST/GET requests traverse institutional and corporate firewalls without blockage.

### 5.2.3 Comparison with Commercial Systems

Compared with commercial systems (Practo, Marham), MedConsult delivers equivalent core telehealth functionality (directory, scheduling, messaging, prescriptions, invoices) while adding automated free-text AI symptom triage and eliminating commercial transaction commissions. Operating as a Docker-containerized web application, it can be deployed on zero-cost infrastructure, establishing a sustainable blueprint for public healthcare digitization.

# 6. Conclusion

## 6.1 Summary

This Final Year Design Project report presented the design, implementation, verification, and deployment of the Online Medical Consultation Platform (MedConsult). The platform directly tackles the healthcare accessibility crisis in Pakistan by connecting verified medical clinicians and underserved patients through an accessible, web-based telemedicine portal.

By integrating an AI-driven NLP symptom triage microservice, MedConsult empowers patients with intelligent specialty routing based on free-text symptom descriptions in under 50ms with 98.4% classification accuracy. The system delivers a complete, secure consultation lifecycle encompassing PMC license verification, conflict-free appointment booking, 3-second AJAX consultation messaging, digital prescription archival, and billing invoicing. Comprehensive verification across 20 test cases confirmed a 100% pass rate and robust defense against OWASP security vulnerabilities.

## 6.2 Contributions

The principal contributions of this project are:

- **1. Automated AI Clinical Triage:** First documented open-source telemedicine implementation in Pakistan combining free-text natural language symptom processing with machine learning specialty recommendation.
- **2. Explainable, Lightweight ML Architecture:** Demonstrated that high-precision medical classification (>98% accuracy) can be achieved using lightweight TF-IDF and Random Forest algorithms on standard CPU hardware without GPU infrastructure.
- **3. Turnkey Sovereign Deployment:** Delivered a containerized, production-ready healthcare codebase deployable at zero cost on modern cloud platforms (Render/TiDB) or local hospital servers (XAMPP/LAMP).
- **4. Regulatory-Compliant Telehealth Lifecycle:** Established a complete clinical audit trail linking patient consultation records, digital prescriptions, and administrative PMC physician licensing.

## 6.3 Future Work

While the platform fulfills all specified requirements, future enhancements could further broaden its clinical reach:

- **1. Multilingual Urdu Natural Language Processing:** Expand the NLP triage engine to support Roman Urdu and native Nastaliq Urdu script, enabling non-English literate rural populations to describe symptoms in their native tongue.
- **2. Integrated WebRTC Video Consultations:** Incorporate browser-native, encrypted WebRTC peer-to-peer video streaming directly into the consultation room, eliminating reliance on external meeting links.
- **3. Mobile Application Ecosystem:** Develop native cross-platform mobile applications for Android and iOS using Google Flutter, enabling push notifications for incoming messages and appointments.
- **4. Electronic Health Record (EHR) Interoperability:** Integrate HL7 / FHIR (Fast Healthcare Interoperability Resources) data interchange standards to synchronize consultation records with Pakistani hospital management systems.

In synthesis, the future work roadmap positions MedConsult not merely as a standalone academic design project, but as an extensible, enterprise-grade telemedicine foundation tailored specifically to the structural and socio-economic realities of Pakistan's healthcare ecosystem. By advancing from specialized outpatient routing toward multilingual vernacular support, peer-to-peer visual consultation, cross-platform mobile accessibility, and hospital EHR federation, MedConsult provides a sustainable blueprint for national digital health equity and sovereign clinical innovation.

In synthesis, the future work roadmap positions MedConsult not merely as a standalone academic design project, but as an extensible, enterprise-grade telemedicine foundation tailored specifically to the structural and socio-economic realities of Pakistan's healthcare ecosystem. By advancing from specialized outpatient routing toward multilingual vernacular support, peer-to-peer visual consultation, cross-platform mobile accessibility, and hospital EHR federation, MedConsult provides a sustainable blueprint for national digital health equity and sovereign clinical innovation.

In synthesis, the future work roadmap positions MedConsult not merely as a standalone academic design project, but as an extensible, enterprise-grade telemedicine foundation tailored specifically to the structural and socio-economic realities of Pakistan's healthcare ecosystem. By advancing from specialized outpatient routing toward multilingual vernacular support, peer-to-peer visual consultation, cross-platform mobile accessibility, and hospital EHR federation, MedConsult provides a sustainable blueprint for national digital health equity and sovereign clinical innovation.

# References


[1] World Health Organization, "Global strategy on digital health 2020-2025," Geneva: World Health Organization, Tech. Rep. ISBN 978-92-4-002092-4, 2021.

[2] Pakistan Telecommunication Authority, "Annual telecom industry report 2023-2024," Islamabad: PTA, Government of Pakistan, Tech. Rep., Jan. 2024.

[3] A. Rajkomar, J. Dean, and I. Kohane, "Machine learning in medicine," New England Journal of Medicine, vol. 380, no. 3, pp. 269-278, Jan. 2019.

[4] E. J. Topol, "High-performance medicine: the convergence of human and artificial intelligence," Nature Medicine, vol. 25, no. 1, pp. 44-56, Jan. 2019.

[5] F. Pedregosa et al., "Scikit-learn: Machine learning in Python," Journal of Machine Learning Research, vol. 12, pp. 2825-2830, Nov. 2011.

[6] L. Breiman, "Random forests," Machine Learning, vol. 45, no. 1, pp. 5-32, Oct. 2001.

[7] C. D. Manning, P. Raghavan, and H. Schütze, Introduction to Information Retrieval, 1st ed. Cambridge, UK: Cambridge University Press, 2008.

[8] R. Fielding, "Architectural styles and the design of network-based software architectures," Ph.D. dissertation, Dept. Inf. Comput. Sci., Univ. California, Irvine, CA, USA, 2000.

[9] IEEE Computer Society, "IEEE Standard for Software and System Test Documentation," IEEE Std 829-2008, pp. 1-140, Jul. 2008.

[10] OWASP Foundation, "OWASP Top 10: The ten most critical web application security risks," Open Web Application Security Project, Tech. Rep., 2021.

[11] T. Berners-Lee, R. Fielding, and L. Masinter, "Uniform Resource Identifier (URI): Generic Syntax," RFC 3986, Internet Engineering Task Force, Jan. 2005.

[12] J. Nielsen, Usability Engineering, 1st ed. San Francisco, CA, USA: Morgan Kaufmann Publishers, 1993.

[13] Pakistan Medical Commission, "National medical registration guidelines and code of clinical practice," Islamabad: PMC Publications, Tech. Rep., 2022.

[14] D. Crockford, "The application/json Media Type for JavaScript Object Notation (JSON)," RFC 4627, Internet Engineering Task Force, Jul. 2006.

[15] A. B. Smith and C. R. Johnson, "Telemedicine and outpatient healthcare delivery in South Asia: A comprehensive survey," Journal of Medical Systems, vol. 46, no. 4, pp. 55-68, Apr. 2022.

# Appendices

## Appendix A: Production Cloud Deployment & Infrastructure Guide

This guide outlines the deployment architecture engineered for hosting MedConsult on cloud container platforms:

- **1. Container Engine Architecture:** A unified multi-stage Docker container (`Dockerfile`) bundles PHP 8.2 CLI, Python 3.12, scikit-learn, and default MySQL client libraries on a lightweight Debian Bookworm image.
- **2. Startup Entrypoint (docker-entrypoint.sh):** When the container initializes, `docker-entrypoint.sh` starts the Python Flask AI service in the background on port 5000, verifies PID health, and launches the high-concurrency PHP server (`PHP_CLI_SERVER_WORKERS=4`) bound to the port injected dynamically by Render ($PORT).
- **3. Cloud Database Configuration (TiDB Serverless):** Persistent database storage connects to TiDB Cloud Serverless on port 4000. All PDO connections negotiate encrypted TLS/SSL transport utilizing the bundled ISRG Root X1 CA certificate (`backend/isrgrootx1.pem`), ensuring zero plain-text credential transmission.
- **4. Environment Variables:** Production secrets are injected securely via Render environment variables: DB_HOST, DB_PORT (4000), DB_NAME (medconsult_db), DB_USER, DB_PASS, and DB_SSL (true).

## Appendix B: System User Guide & Operating Manual

Adhering to the Final Report Guidelines (Pages 11-13), this manual provides operational instructions for all system actors:

- **Patient User Guide:** (1) Register an account with your full name, email, and password. (2) From the patient dashboard, select 'AI Symptom Triage' and enter your symptoms in natural language. (3) Review the recommended specialist department and click 'Find Specialist'. (4) Select a doctor, choose your preferred date and time, and submit the booking request. (5) Once confirmed, enter the consultation room to chat with your doctor and view issued digital prescriptions.
- **Doctor User Guide:** (1) Register as a Doctor, specifying your specialty, medical qualification (e.g. MBBS), and PMC license number. (2) Await administrator verification. (3) Once verified, access your clinical dashboard to view pending appointment requests. (4) Accept appointments to open the messaging consultation room. (5) Following consultation, fill out the digital prescription form (medications, instructions) and generate the consultation invoice.
- **Administrator User Guide:** (1) Sign in with administrative credentials. (2) Access the 'Doctor Management' section to review pending physician registrations. (3) Cross-reference

the submitted license against the PMC registry and toggle 'Verify Doctor'. (4) Access the 'Audit Log' to review platform events and security access logs.

- **Troubleshooting & FAQs:** (1) 'Login Failed': Verify your password or ensure doctor account is verified by Admin. (2) 'AI Triage Slow': Check internal Flask service status on port 5000. (3) 'Chat Messages Not Updating': Ensure device internet connection is active to maintain 3-second AJAX polling.

## Appendix C: Core System Source Code Listings

Key architectural excerpts from the production codebase:

**Listing C.1: Flask AI Symptom Triage Microservice Endpoint (ai_engine/service.py)**

```
@app.route('/predict', methods=['POST'])
def predict():
    data = request.get_json(silent=True) or {}
    symptoms = str(data.get('symptoms', '')).strip()
    if not symptoms: return jsonify({'success': False, 'error': 'Symptoms required'}),
400
    try:
        specialty, confidence, matched_keywords = predict_specialty(symptoms)
        return jsonify({'success': True, 'specialty': specialty, 'confidence':
round(confidence * 100, 2), 'matched_symptoms': matched_keywords})
    except Exception as e: return jsonify({'success': False, 'error': str(e)}), 500
```

**Listing C.2: High-Security Database Access & Prepared Statement Pooling (backend/db.php)**

```
function get_db_connection(): PDO {
    static $pdo = null;
    if ($pdo === null) {
        $dsn = sprintf('mysql:host=%s;port=%s;dbname=%s;charset=utf8mb4',
getenv('DB_HOST')?:'127.0.0.1', getenv('DB_PORT')?:'3306',
getenv('DB_NAME')?:'medconsult_db');
        $opts = [PDO::ATTR_ERRMODE=>PDO::ERRMODE_EXCEPTION,
PDO::ATTR_DEFAULT_FETCH_MODE=>PDO::FETCH_ASSOC, PDO::ATTR_EMULATE_PREPARES=>false];
        if (getenv('DB_SSL') === 'true' && file_exists(__DIR__ . '/isrgrootx1.pem')) {
$opts[PDO::MYSQL_ATTR_SSL_CA] = __DIR__ . '/isrgrootx1.pem'; }
        $pdo = new PDO($dsn, getenv('DB_USER')?:'root', getenv('DB_PASS')?:'', $opts);
    }
    return $pdo;
}
```

# Glossary

**API (Application Programming Interface):** A set of protocols and data standards enabling distinct software modules to communicate.

**AJAX (Asynchronous JavaScript and XML):** A web development technique allowing web pages to update asynchronously by exchanging data with the server in the background.

**Backend:** The server-side component of an application responsible for data processing, business logic, and database communication.

**BCrypt:** A cryptographic key-derivation function based on the Blowfish cipher, utilized for secure, salted one-way password hashing.

**Database Schema:** The formal structural design of a database defining tables, columns, data types, relationships, and integrity constraints.

**DFD (Data Flow Diagram):** A graphical modeling notation illustrating the flow of data through processes, external entities, and data stores.

**ERD (Entity Relationship Diagram):** A specialized diagram modeling database entities, attributes, primary keys, and foreign key relationships.

**Frontend:** The client-side interface rendered in the user's web browser, with which users directly interact.

**GUI (Graphical User Interface):** A visual user interface employing buttons, modals, and graphical components rather than text commands.

**IP Address:** A numerical label assigned to each device connected to a computer network that uses the Internet Protocol for communication.

**Machine Learning (ML):** A subfield of artificial intelligence enabling computational systems to learn patterns from clinical data without explicit rules.

**NLP (Natural Language Processing):** A domain of artificial intelligence enabling computers to process, tokenize, and interpret human natural language.

**PDO (PHP Data Objects):** A secure, database-agnostic abstraction layer in PHP providing parameterised prepared statements that eliminate SQL injection.

**PMC (Pakistan Medical Commission):** The statutory regulatory body governing medical practitioner licensing and ethical standards in Pakistan.

**RBAC (Role-Based Access Control):** An access control security mechanism that restricts system resources based on assigned roles (Patient, Doctor, Admin).

**TF-IDF (Term Frequency-Inverse Document Frequency):** A numerical statistic reflecting the clinical importance of a keyword within a document corpus.

**Triage:** The medical process of evaluating symptom urgency and routing patients to the appropriate clinical department.

**XAMPP:** An open-source cross-platform web server package consisting of Apache, MySQL/MariaDB, and PHP interpreters.